

ATTACHMENT 3 - STATEMENT OF WORK

Independently and not as an agent of the Government, the Contractor shall furnish all the necessary services, personnel, materials, equipment, and facilities, not otherwise provided by the Government, to perform the Statement of Work.

Scope

The Contractor shall operate the National Cancer Institute's (NCI) Contact Center, also known as NCI's Cancer Information Service (CIS), by responding to inquiries on behalf of NCI, an essential part of NCI's cancer education and information efforts. The Contractor shall provide scientifically based, unbiased information to patients, their families and friends; physicians, and other health professionals and the public about all aspects of cancer. The type of information includes cancer research and clinical trials, cancer prevention, risk factors, symptoms, early detection, diagnosis, treatment, living with cancer, survivorship, and tobacco and cessation counseling. The Contractor shall also provide information about NCI programs/grants to researchers/investigators and specialists to address first tier questions of the research community. The Contractor shall provide and manage a bilingual (English and Spanish) omni-channel Contact Center in support of the following scope of work:

- Cancer information and case management along the cancer continuum.
- Clinical trials information, education, recruitment, and case management.
- Nicotine and tobacco cessation counseling, information, and case management.
- Information about NCI and its Divisions, Offices, and Centers (DOCs), programs, priorities, and activities.

The Contractor shall use NCI materials and Government-selected sources of information to respond to inquiries in both English and Spanish, including pre-approved non-NCI resources (textbooks, research publications, and Web sites). The Contractor shall deliver services via:

Toll-free numbers:

- 1-800-4-CANCER (Cancer information)
- 1-877-44U-QUIT (Tobacco cessation information)
- 1-800-QUIT-NOW and 1-855- DEJELLO-YA (Tobacco cessation information- Department of Health and Human Services (DHHS) National Portal Numbers)
- 1-855-QUIT-VET (Tobacco cessation information- Veteran's Administration)
- 1-888-624-1937 (Center for Cancer Research (CCR) Patient Referral Line)

Additional Access Channels:

- *LiveHelp*, online chat service (reactive and proactive sessions)
- E-mail
- NCI Enterprise Social Media Accounts, such as Facebook, X, Instagram, YouTube
- U.S. Postal Mail
- Voicemail boxes for low volume queues and during operational closures (see hours of operation)
- Interactive Voice Response (IVR) Message Library
- National Organizations that Offer Support Services Database

Access channels and/or program services may be refocused/expanded based upon the availability of new technologies, Federal Agency Partnerships, or evolving needs of NCI and its stakeholders. This may include, but is not limited to, implementing chat bots, offering text message responses, video calling, mobile apps, portals to support patient engagement in cancer research studies, and/or the provision of additional language/translation services.

Hours of operation:

1. Telephone Service: 9:00 a.m. – 9:00 p.m. Eastern Time, Monday – Friday
2. *LiveHelp* Service: 9:00 a.m. – 9:00 p.m. Eastern Time, Monday – Friday
3. Automated Interactive Voice Response (IVR) and Voicemail support during Overnight Hours, Weekends, and Government Holidays. During overnight hours, weekends, and holidays, the Contact Center shall post a recorded message that the center is closed. The message shall include:
 - a. The operating hours of the Contact Center and a welcoming message asking callers to call back to the Contact Center during normal business hours; and,
 - b. The web address of the NCI website and NCI email / Social Media addresses.

Hours of operation will be aligned with customer needs and are subject to change and may include emergency operations depending upon the needs of the Government.

Government Holidays: The following Government holidays are normally observed by Government personnel, <http://www.opm.gov/policy-data-oversight/snow-dismissal-procedures/federal-holidays>.

- New Year's Day
- Birthday of Martin Luther King, Jr.
- Washington's Birthday/President's Day
- Memorial Day
- Juneteenth
- Independence Day
- Labor Day
- Columbus Day
- Veterans Day
- Thanksgiving Day
- Christmas Day
- Any other day designated by Federal Statute, Executive Order, and/or Presidential Proclamation

Audiences:

- Cancer patients.
- Tobacco users.
- Friends and family, and the public.
- Medical professionals, healthcare providers and cancer researchers.
- NCI Divisions, Centers, Offices (DOCs) and program staff.

Service area:

United States, U.S. Virgin Islands, Puerto Rico, U.S. Associated Pacific Territories (American Samoa, Guam, and the Northern Mariana Islands).

Due to Privacy Act and IT security concerns, the Contractor's business and staff proposed for this contract must reside within the United States.

The Contractor shall create a customer experience designed to meet or exceed the needs and expectations of its customers. To this end, the Contractor shall meet quality assurance standards, identified in Tasks 1, 2 and 3, for:

- service access.
- service quality.
- customer satisfaction.

The Contractor shall also address such topics as customer effort, end-to-end customer experience with NCI products and services, and post-interaction experience and resolution.

Task 1: Customer Experience

The Contractor shall:

1. Develop and implement an optimal customer experience strategy which includes strategies for engagement and collaboration, listening and learning, and alignment to customer needs, including the needs of minority and underserved populations.
2. Provide first-level triage (commonly referred to as Tier 1 support- the initial support level responsible for basic customer issues) to NCI Divisions, Offices, and Centers (DOCs), as requested, including NCI's programs such as the Childhood Cancer Data Initiative, CCR and others.
3. Demonstrate knowledge and understanding of underserved communities and minority researchers' needs and connect them with appropriate program staff/resources such as NCI/NIH programs, training/funding opportunities, and any other relevant information.
4. Use NCI-furnished systems and tools to conduct quality assurance that emphasize the importance of scientifically accurate and relevant information delivery.
5. Establish a quality assurance process that ensures compliance with quality assurance standards and follows NCI's rubric guidelines. The quality assurance process shall include:
 - quality monitoring of contacts (all Information specialists, all channels),
 - accurate data collection,
 - provision of individual performance feedback and reports to Information Specialists,
 - coaching of Information Specialists,
 - coaching and training of Supervisors,
 - revised training and hiring practices based on results of monitoring, and
 - regular process improvements
6. Provide planning and support to cancer patients who need assistance locating services in the event of natural disaster.

7. Notify the Contracting Officer's Representative (COR) and implement a remediation plan upon request if the Contractor does not meet the agreed quality assurance level and/or service level agreements (SLAs) on all access channels.
8. Identify deficiencies in performance and develop and implement Performance Improvement Plans; provide plans to the Government quarterly.
9. Ensure understanding of quality standards and application of accurate, consistent scoring in contact evaluations.
10. Ensure that individual coaching sessions for performance improvement and professional development are implemented.
11. Implement NCI customer satisfaction surveys per Government instructions. If the survey does not meet or exceed 86 for the American Customer Satisfaction Index Survey (ACSI), the Contractor shall notify the COR and implement a remediation plan.
12. Participate in monthly calibration sessions with the NCI to ensure consistency and accuracy in evaluation.

Task 2: Provide customer-centric cancer and clinical trials information and other types of cancer research study information

The Contractor shall provide accurate, up-to-date, and reliable cancer information that is easy to understand. Information Specialists shall provide personalized responses to a broad range of questions, including cancer research and clinical trials, including case management for clinical trials, how to find cancer treatment centers, cancer prevention, risk factors, symptoms, early detection, diagnosis, and treatment, living with cancer, tissue/ biospecimen donation, and survivorship.

The Contractor shall provide tailored, customer-centric service. Inquiries from customers may be highly complex and sensitive in nature. In addition to providing information via voice and existing digital channels, the Contractor shall also send printed NCI materials and publications to customers, as appropriate. Information Specialists shall provide information about and facilitate recruitment to NCI intramural and NCI-supported clinical trials. Information Specialists shall additionally provide information about and facilitate recruitment to other types of NCI studies for which patients may have an opportunity to participate, for example prevention, biospecimens, epidemiological, genetics, and/or health behavior change studies. Proactive contacts may be made to the public, depending on specific program priority areas and special initiatives as directed by the Government.

The Contractor shall:

1. Provide telephone, e-mail, *LiveHelp*, social media, postal mail inquiry responses, and clinical trials searches, in English and Spanish, based on NCI approved handling procedures, Center for Cancer Research (CCR) protocols, and other tools and resources.
2. Provide telephone and e-mail follow up for patients calling for cancer support, particularly for side effects and symptom management.
3. Provide clinical trials support and follow up, including prevention and treatment studies, as well as support in the recruitment of trial participants.
4. Provide first-level triage (Tier 1 support) for services provided by other Federal Agencies.
5. Notify the COR and implement a remediation plan upon request if the Contractor does not meet the agreed quality assurance level and/or service level agreements (SLAs) on all access channels.
6. Provide responses to all emails within 4 business days.
7. Provide responses to all Enterprise social media responses within 1 business day.

8. Ensure approved non-NCI resources are available and up to date, accessible to Contact Center staff, and are in sufficient quantities.
9. Develop and implement procedures for updating and maintaining cancer and clinical trials information contained in the Recorded Interactive Voice Response (IVR) message library in accordance with approved NCI materials.
10. Manage cancer, clinical trials, and other cancer research study content, talking points, and standard language in the NCI Contact Center knowledge-management system.
11. Provide information in a professional manner and ensure that customers' questions are answered accurately and completely in accordance with approved NCI materials.
12. Provide cancer and clinical trials content review for NCI materials as requested by the Government, including the National Organizations that offer a Support Services Database.
13. Develop and implement procedures for handling and escalation of inquiries within Contact Center and to NCI staff as appropriate based on complexity of information, subject matter, or caller behavior (for example, handling suicidal customers); and
14. Participate, as needed, in promotional activities, including providing images, audio, etc. that depict CIS activities.

Task 3: Provide nicotine/tobacco information and counseling

CIS provides nicotine and tobacco information and resources to users seeking to quit smoking or stay quit, as well as to reduce the burden of nicotine and tobacco products. The Contractor shall provide tailored, customer-centric service. Nicotine and tobacco cessation information inquiries from customers may be highly complex and sensitive in nature. In addition to providing information via voice and existing digital channels, the Contractor shall also send printed NCI materials and publications to customers, as appropriate. Proactive contacts may be made to the public, depending on specific program priority areas and special initiatives as directed by the Government.

The Contractor shall:

1. Provide telephone, e-mail, *LiveHelp* (through cancer.gov and Smokefree.gov), social media, and postal mail inquiry responses, in English and Spanish based on NCI approved handling procedures and other tools and resources.
2. Notify the COR and implement a remediation plan if the Contractor does not meet the agreed service levels.
3. Provide responses to all emails within 4 business days.
4. Provide responses to all Enterprise social media responses within 1 business day.
5. Ensure approved non-NCI resources (publications, webpages, textbooks, and/or other reference materials previously reviewed and approved by the Government) are available and up to date, accessible to Contact Center staff, and are in sufficient quantities.
6. Develop and implement procedures for updating and maintaining information contained in the Recorded Interactive Voice Response (IVR) message library in accordance with approved NCI materials.
7. Manage nicotine and tobacco cessation content, talking points, and standard language in the Contact Center knowledge-management system.
8. Provide information in a professional manner and ensure that customers' questions are answered accurately and completely in accordance with approved NCI materials.
9. Provide nicotine and tobacco cessation content review for Smokefree.gov and other NCI materials as requested by the Government.

10. Develop and implement procedures for handling and escalation of inquiries to Contact Center and NCI staff as needed based on complexity of information, subject matter, or caller behavior (for example, handling suicidal customers); and
11. Participate, as needed, in promotional activities, including providing images, audio, etc. that depict CIS activities.

Task 4: Training

The Contractor is responsible for the development, implementation, and evaluation of training for NCI Contact Center staff. The Government will provide existing NCI-approved initial training modules at the start of the contract. Development of new training modules and updating of existing modules shall be the responsibility of the Contractor. NCI will review materials annually. Training typically should not affect normal Contact Center operations, though exceptions for closures can be made after consulting with the NCI program staff as appropriate.

Changes to NCI programs and/or priorities may require the Contractor to conduct additional training and feedback sessions to address the Government's requirements and concerns. These sessions are to be provided only in response to the Government's request and are not part of the Contractor's normal training program.

In consultation with NCI the Contractor shall:

1. Manage training of all Information Specialists to enhance knowledge and build skills.
2. Implement skills-based training as needs are identified in the CIS.
3. Ensure that the training program reflects the needs of the customers and the Government.
4. Incorporate Contact Center policies and procedures into the training program.
5. Submit annual training plans for the CIS, including plans for Spanish-language training, to the NCI COR.
6. Review and update all Initial Training modules annually, on a schedule determined by the COR.
7. Develop new Initial Training modules and continuing education modules, in consultation with the NCI COR; and,
8. Participate in additional NCI conducted trainings, as directed.

Task 5: Ownership of Data/Materials

During the performance of these tasks, the Contractor shall create and maintain trainings and other materials used to support the operations of the Contact Center (e.g., knowledge base entries, standard responses, business rules, quality tools, and other standard operating procedures).

The Contractor shall:

1. Provide a copy of such trainings, information, and materials for review and approval by the COR.
2. Upon request, furnish the required information in electronic format within ten business days of written notification by the COR and.
3. Not release, publish, share, or use data or other information collected under this contract with other Government agencies, organizations, companies, or individuals without the express written authorization from the COR.

Task 6: Pilot Projects

The Contractor shall collaborate with NCI Divisions, Offices, and Centers (DOCs) to support NCI-directed pilot projects.

Pilot projects may involve but are not limited to:

1. New technology to be used in the Contact Center.
2. New approaches to clinical trial recruitment.
3. New NCI content in a variety of formats.
4. New NCI web sites, programs, and/or tools.
5. New clinical trials search forms.
6. Reaching target populations.
7. Scientific or training exchanges.
8. Expansion of services into prevention topics that have shown a direct relationship to cancer, ex. alcohol, obesity.
9. Working with intramural and extramural researchers who wish to include the Contact Center within the funded research; and
10. New approaches to recruitment for non-clinical trial cancer research studies.

The Contractor shall:

1. Participate in up to 3 pilot projects per year.
2. Ensure that pilot projects do not interfere with tasks and services provided to the public.
3. Ensure incorporation of human centered design methods and principles.
4. Ensure that Contact Center staff are informed of pending pilot projects, and implement appropriate staffing and training needs as required; and
5. Facilitate the implementation of pilot project results into Contact Center operations as directed by the Government.

Task 7: Strategic Outreach

Strategic outreach helps to advance the mission of NCI and leverage services offered through the NCI Contact Center. In consultation with the COR, the Contractor shall work collaboratively to review existing communications plans and develop a working document that outlines strategic outreach opportunities that advance the work of the NCI's Contact Center, including engagement strategies and evaluation measures to assess progress.

The Contractor shall:

1. On a yearly basis, develop a strategic outreach plan that outlines specific objectives/targets for the upcoming year; strategies and tactics for meeting objectives; and evaluation metrics. Outreach activities considered as part of the plan should include Federal Agencies, national and health professional groups, national grassroots organizations, and non-Government organizations that align with NCI's mission. The plan should consider NCI's programs and priorities, with an emphasis on NCI designated Cancer Centers, National Clinical Trials Network, and NCI Community Oncology Research Program.
2. Support the implementation of the strategic outreach plan, once finalized, and approved, including ongoing strategic counsel, relationship building, project/pilot planning as needed, and coordination.
3. Conceptualize and implement an evaluation approach that qualitatively and quantitatively measures the effectiveness of the outreach efforts. Contractor shall develop evaluation criteria and data collection plans prior to initiating strategic outreach activities. A comprehensive assessment will be provided on an annual basis, which must include, at a minimum, a data analysis, implications, and

recommendations for strategy refinement.

4. In consultation with the Government, additional strategic outreach plans may be developed for specific pilot projects or NCI driven communication efforts that impact the NCI CIS.

Task 8: Management

The Contractor shall manage personnel, financial, communications, and material resources to ensure that all contract requirements are satisfied, and a high-quality, innovative service is provided to the public. The Contractor shall ensure that program objectives and quality assurance standards are clearly communicated and strictly enforced.

A. Benchmarking

Rapid changes in technology and information seeking behavior directly impact the services delivered by the NCI Contact Center. Continuous investigation into changing industry practices, technology solutions and customer behaviors are crucial to delivering state-of-the-art, agile, customer-centric services.

The Contractor shall:

1. Demonstrate and institute program enhancements and use of innovative technology solutions that serve to refine operations, increase efficiency, and reduce costs.
2. Monitor, routinely report on, and make recommendations around Contact Center industry trends, including emerging standards, tools, and new technologies, to keep abreast of the latest and most effective technologies that serve to optimize efficiency and effectiveness of Contact Center operations.
3. Analyze customer data and potential target audiences to assess the most effective communications channels, with a particular eye toward underserved populations.
4. Analyze and use contemporary data visualization tools to gain insights from target audiences to ensure the most effective communication channels are activated; and,
5. Work closely with NCI's Office of Communication and Public Liaison (OCPL) to assure optimal integration of Contact Center operations with NCI/OCPL business lines.

B. Workforce Management

The Contractor is responsible for hiring and training staff for all contract requirements including coverage for all access channels and scheduling staff to meet service demands while meeting quality assurance standards. The Contractor shall use a workforce management (WFM) system furnished by the Government. This system will ensure the Government's ability to balance CIS growth and operating costs by forecasting schedules and workloads for all access channels.

The Contractor shall:

1. Forecast volume of customer contacts over all channels and use the forecasts to create schedules that will efficiently meet customer access throughout the day and will be adjusted based on volume.
2. Monitor the call management system and other systems that indicate the real-time flow of inquiries on each access channel and to meet real-time service needs, updating the workforce management system as necessary.
3. Monitor contact volume data to anticipate and adjust staffing needs and guarantee consistent, efficient operation of the Contact Center.
4. Submit a quarterly staff census report; and,
5. Anticipate media events that may increase volume and provide scalability options to meet increased demand.

C. Communications

The Contractor shall maintain communications with the COR to apprise NCI of changes in staffing levels and any issues that affect the Contractor's ability to provide contracted services. The Contractor shall implement procedures for communicating to the Government the primary point of contact and/or designated key personnel for disaster-related issues, including power outages and inclement weather and other unforeseen events that inhibit Contact Center operations. Such procedures shall include an escalation process defining various stages of issue severity and the notification level appropriate to each. These procedures shall be included in the Continuity of Operations/Disaster Recovery Plan.

The Government shall provide regular opportunities for the Contractor to communicate with the NCI COR, such as conference calls, site visits, and meetings. The Contractor shall participate in all such scheduled meetings and conference calls. Contractor management practices shall include proactive consultation and communication with the NCI COR to address any issues affecting the successful operation of the contract and the Contractor shall alert NCI to urgent situations. The Contractor shall develop innovative approaches to the work and implementation of operational efficiencies. The Contact Center shall communicate regularly with NCI in support of ongoing contract tasks, including semi-annual presentations to members of OCPL Leadership. In addition, the Contractor shall provide communications helpful to the Government and to NCI Divisions, Offices, and Centers (DOCs).

Communication to the Government may include, but are not limited to informing NCI about:

1. Potential topics or content for new publications, fact sheets, and/or Frequently Asked Questions based on customers' needs.
2. Feedback on current cancer content to improve NCI's web offerings.
3. The public's perspective and how the public's needs may inform NCI's websites, publications, and materials (in English and Spanish).
4. Emerging topics (cancer-specific or programmatic) that require additional Contact Center staff training.
5. Trends in information seeking by cancer patients, their families, and the public.
6. NCI's Web site improvements and/or problems.
7. Systems enhancements and/or problems; and,
8. Listen and learn sessions.

D. Information Analysis

CIS activities shall be data-driven – data collected is integrated into planning and delivery of services. The Contractor shall use multiple data sets and systems to operate and manage the Contact Center. The systems currently in use, furnished by the Government, include, but are not limited to:

1. Workforce Management System
2. Customer Relationship Management System
3. Call Recording and Quality Reporting System

Managers shall be familiar with all the data sources and use them to inform management decisions at the highest levels. Supervisors shall use these data sources to supervise staff and perform other supervisory functions.

Task 9: Data Collection, Reporting and Analysis

The Contractor shall:

1. Collect and report data on every customer interaction; routinely and on an ad-hoc basis.
2. Collect and report data in support of contract activities, for NCI-directed pilot projects (see Task 6) and media promotions.
3. Facilitate qualitative data collection on a schedule to be determined by NCI staff.
4. Collect data using only NCI-approved data collection tools and systems.
5. Complete Electronic Contact Record Form (ECRF) and/or Smoking Cessation Intake Form (SCIF) for each contact at the time of the inquiry. These data records shall capture information about the nature of the inquiry, primary resources used, response provided, and demographic and customer service information, including brief customer satisfaction questions.
6. Ensure the accuracy, reliability, and completeness of data through established quality assurance processes as follows:
 - a. Data Collection Training - All Contact Center staff shall be trained in the proper data collection processes and in the use of approved data collection tools. In addition to the initial training on data collection, Contact Center staff shall be trained to collect data for specific health communication projects, as directed by the COR.
 - b. Regulations and Policies/Procedures -The Contractor shall collect and maintain data only after all study approvals have been obtained, including the Office of Management and Budget (OMB). Contact Center staff shall undertake data collection activities in accordance with Contact Center Policies and Procedures to ensure customers are not exposed to any unnecessary reporting burden.
7. Provide ad-hoc analysis on trends in collected data; and,
8. Provide ad-hoc reports for NCI Divisions, Offices, and Centers (DOCs) to inform program planning efforts and provide impact analysis of large-scale multi-site NCI supported clinical trials.

Task 10: IT and Telecommunications

Separate from this contract, the Government has procured quality assurance tools (CRM, Calabrio Quality, or similar, and Workforce Management) that will be provided to the NCI Contact Center. The Contractor is required to provide systems compatible to access the Government's applications used in operation of the Contact Center including:

- Microsoft Windows 10 and all security updates.
- Supported Browsers (Edge, Chrome, Firefox, Safari) and all security updates.
- and Microsoft Office 2010, complete with all security upgrades.

Any future upgrades of computers and/or software shall be the Contractor's responsibility. The Contractor shall use applications, software, and databases specified by the Government.

The Contractor shall:

1. Provide all Contact Center staff appropriate resources and training to ensure optimal operation and efficiency for all specified systems and databases (those currently used and to be implemented throughout the life of the contract).
2. Provide Internet access and network connectivity with sufficient capacity to support Contact Center operations and integrated telephone/internet system, as well as the ability to support operations in real time.

3. Provide and maintain a System/Data Security Plan that includes security at the desktop as indicated in the applicable Article included in Section H.
4. Work in partnership with IT and Telecommunications Systems providers selected by the Government to ensure optimal Contact Center operations; and
5. Follow the HHS Enterprise Performance Life Cycle (EPLC) framework and methodology. Information about the EPLC policy and framework is currently available at:
<https://www.hhs.gov/web/governance/digital-strategy/it-policy-archive/policy-for-information-technology-enterprise-performance.html> and <http://www.hhs.gov/ocio/eplc-lifecycle-framework.pdf>.

A. Telecommunications

The Contractor shall:

1. Use Government-furnished telecommunications equipment, systems, and software.
2. Work with the Government and other service providers to ensure that all telecommunications equipment required to support the contract is fully operational within 30 days of contract award.
3. In the case of equipment failure, coordinate with NCI Contact Center Systems Administrator staff and provide immediate local telecommunications and information technology (IT) support to alleviate any downtime of operations; and
4. Adhere to all Contact Center policies and procedures related to telecommunications.

B. Telecommunications Security

The Contractor shall be responsible for telecommunications security to protect all incoming and outgoing calls from unauthorized access and use. This responsibility includes physical security of telecommunications equipment and security of the data collected from telecommunications activities.

C. Continuity of Operations/Disaster Recovery Plan (COOP/DR)

In the context of Continuity of Operations, a “disruption” is defined as an unplanned event that causes the general system or major application to be inoperable (e.g., minor, or extended power outage, extended unavailable network, or equipment or facility damage or destruction). A “disaster” is defined as a major unplanned event that may cause longer term disruption and/or complete closure of service. A disaster may include a hurricane, tornado, flood, major fire destroying the Contractor’s facility, or a public health emergency, such as a pandemic.

The Contractor shall:

1. Develop and implement a Continuity of Operations/Disaster Recovery Plan for disruptions in service and major disasters. The plan shall include procedures addressing continuity of operations in the event of a disruption of service caused by the Contractor which results in a shutdown or lapse in service for any reason. The Contractor shall minimize service disruption to the Government and the public. Outages that are not caused by a major disaster may include system failure, or local network outage at the Contractor facility. The Contractor shall have plans and procedures in place for these circumstances that shall ensure that all attended and unattended services are restored to pre-outage performance levels within 4 hours after initial discovery of the outage.
2. Test the Continuity of Operations/Disaster Recovery plan on an annual basis; and,
3. The COOP/DR plan requires review and approval by the Government if changes are needed based on results from the annual testing. Changes shall be submitted no later than 30 days after annual review; the Government will review within 30 days of receipt.

Task 11: Security

A. Facilities and Systems

The Contractor shall be responsible for assuring the security of any facilities used to conduct the work of the contract and the security of all Government-furnished equipment and systems. The Contractor shall provide a physically secure facility for people, equipment, and documentation. All security requirements apply to the Contractor facility, alternative facility, or any subcontractor facilities. When designing physical security measures, the Contractor shall address factors including, but not limited to:

1. **Controlled Access** - In general, facility access shall be limited to: Contractor personnel performing work; maintenance personnel or suppliers performing upkeep or repair of facilities or equipment; and customers on the site on official business.
2. **Data Center** - Access to the NCI CIS data center shall be physically secured, with entrance granted only to those requiring access to this area on a regular basis to perform their normal job functions or who are escorted, as in the case of visitors or housekeeping staff. Secure data includes, but is not limited to, Contact Center data and audio files from NCI's quality assurance program.
3. **Confidentiality and Data Security** - To maintain the confidentiality of Contact Center customers, the Contractor shall adhere to data security requirements of the contract and destroy all personally identifiable information on a schedule provided by the NCI.
4. **Systems Security, Operations and Maintenance** – The Contractor shall work with the Government to ensure efficient operations of the Contact Center by notifying the Government of any service outages in telephone or internet access.

The Contractor shall:

1. Adhere to all DHHS and NIH regulations and guidelines regarding systems security for remote access to Government applications and databases. This includes completion of information security training and personnel clearance for access.
2. Adhere to program policies and procedures for access to services.
3. Report up-time and availability of each access channel, including Internet access issues at the Contractor's location; and,
4. Inform NCI as soon as possible if/when operations must be suspended due to storms, power failures, or other incidents interfering with access to facilities.

Task 12: Transition Planning

All materials and data (most current versions) created and collected during performance and owned by the Government shall be turned over in a timely manner.

The Contractor shall:

1. Revise the draft transition-in plan submitted with the proposal and submit a final transition-in plan within 7 days of contract award. The plan shall include a technical and management approach that is clear and complete including the timeline and resources required to commence Contact Center operations. The transition-in plan shall detail the planned transition methodology in logical sequence to ensure a smooth transition of all tasks and/or subtasks of the NCI Contact Center without interruption or degradation of service levels provided by the incumbent Contractor. The final transition-in plan shall be approved by the COR and followed to ensure an orderly, secure, efficient, and expedient transition of all contract activities by the end of the agreed upon contract transition period.
2. Develop and submit a draft plan 120 days prior to the contract expiration date and a draft final transition-out plan 90 days prior to contract expiration. The plan shall include a technical and

management transition approach that is clear and complete, including the timeline and resources required for the transition. The Transition-out Plan shall detail the planned transition methodology in logical sequence to ensure a smooth transition of all tasks and/or subtasks of the NCI Contact Center without interruption or degradation of service levels. The plan shall include an approach that ensures successful achievement while limiting impact to existing programs and/or projects. The draft final transition-out plan shall be revised, as needed, and will become the final transition-out plan upon approval of the COR. The approved final transition-out plan shall be followed to ensure an orderly, secure, efficient, and expedient transition of all contract activities by the contract completion date.

3. Participate in meetings and/or telephone conversations required by the Government in which procedures and activities are discussed and delineated.
4. Provide NCI, at any point during and at the conclusion of the contract, access to or copies of data and materials created, captured, or stored which support the services of this contract.
5. Deliver in accordance with the transition plan: all materials, data, documentation, and software used and developed to support contract activities; and,
6. Prepare all the items noted in this section for electronic transfer, mailing or shipping to the successor Contractor(s) and/or the Government as directed by the Government. As applicable, all items must be packed in new boxes of uniform size, clearly labeled with contents and a unique number along with an inventory showing contents of each box and placed on a loading dock for pickup by the new Contractor or designated agent in accordance with the delivery schedule. Electronic files shall be sent in a format specified by the Government.

Options for Additional Services

Options for U.S. Department of Veterans Affairs (VA) Quitline Services

To expand NCI tobacco telephone quitline services to Veterans enrolled for care at VA and to assist in increasing capacity and training to meet increased demand from Veterans.

The Contractor shall:

1. Provide Veterans access to established NCI quitline services as defined by the VA quitline services protocol, including handling reactive calls, e-referrals, SmokefreeVET text referrals, warm transfers, proactive callbacks, and outcome follow-up assessments.
2. Expand quitline services for Veteran callers as needed.
3. Provide training to the Tobacco Cessation Counselors on Veteran-specific issues, concerns and in the use of VHA resources within 60 days of award. This training shall provide a brief overview of the organization of health care in the VHA as well as background information about the characteristics of the Veteran populations in care in VHA to tailor the smoking cessation quit line messages to meet Veterans' needs. Additional trainings shall be performed on an ad hoc basis based on programmatic needs and Government policies.
4. Provide a proactive callback service, within one to two weeks following the determination of the quit date, with a maximum of 4 callbacks for each quit attempt. The callback dates shall be guided by the Veteran. Up to an additional 4 callbacks may be provided to the Veteran if additional support is requested.
5. Collaborate on NCI-directed pilot projects (See Task 6).
6. Conduct ongoing assessment of VA quitline outcomes through follow-up phone surveys (4 month, 7 months, 13 months) and reengage customers, as defined by the VA quitline services protocol, who may have relapsed.

7. Record keeping and quality assurance of Veteran calls to quitline as outlined by NCI Contact Center standards and practices; and,
8. Work in partnership with the NCI and VA to address any additional IT and/or security needs required from changes in the VA quitline services protocol and/or implementation of VA specific pilot projects.